

Policies and Initiatives:

- “Help Me Quit”(Action on Smoking and Health).
- Tackling Illegal Tobacco (GOV.WALES) (ASH)
- Collaboration and Data Monitoring (GOV.WALES) (Senedd Research)
 - <https://www.gov.wales/tobacco-control-strategy-wales-and-delivery-plan-html>
 - <https://ash.wales/smoke-free-wales-roadmap/>
 - <https://research.senedd.wales/research-articles/a-smoke-free-wales/>

Future Goals:

The ultimate objective is to make Wales smoke-free by 2023 by bringing the prevalence of smoking down to less than 5%.This involves ongoing efforts to denormalize smoking, support cessation, and prevent smoking initiation, especially among young people (GOV.WALES) (Welsh Government News).

Summary:

Smoking is one of the most dangerous activities globally, causing various diseases in individuals. One such illness is chronic obstructive pulmonary disease (COPD), a dangerous lung condition characterized by inflammation of the bronchial tubes.Early detection is essential to stop the further development of this disease. The most efficient method of reducing the risk and progression of COPD is smoking cessation(WHO,2023).

Role Of Nurses:

Nurses are integral to the comprehensive care of adults with COPD caused by smoking. Their roles in patient education, smoking cessation support(NMC Code,section2.3,3.1), symptom management(NMC Code,section6.1,10.1),and psychosocial care(NMC Code,section3.3) are vital for improving patient outcomes and quality of life(NMC Council).

Professionals:

Pulmonologists: Specialists in lung diseases who can provide personalized information and treatment options related to COPD and smoking.

Respiratory Therapists: Healthcare professionals who specialize in treating patients with breathing problems and can offer practical advice on managing COPD and quitting smoking.

Websites/Resources:

- Royal College of Nursing (RCN):
 - The RCN emphasizes the critical role nurses play in smoking cessation. They highlight that stopping smoking is one of the most effective interventions to slow the progression of chronic diseases like COPD. official website:[RCN's Smoking Cessation page] (<https://rcnlearn.rcn.org.uk/Search/Smoking-cessation>).
- Nursing and Midwifery Council (NMC):
 - The NMC sets standards for nursing and midwifery practice, which include promoting public health initiatives like smoking cessation official website:[NMC website](<https://www.nmc.org.uk>).

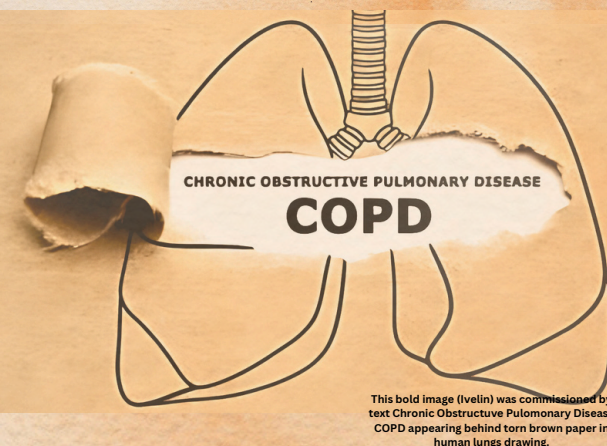
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The Impact of Smoking on Lung Health: Understanding COPD



Target Audience:

- Adult Nurses
- The community Respiratory Team (CRT) which includes Respiratory Nurse Specialists (RNS) and Physiotherapists
- Pulmonologists (lung specialist)
- Tobacco Cessation Specialists

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Definition:

“Chronic Obstructive Pulmonary disease (COPD) is defined as a disease characterized by the presence of airflow obstruction due to chronic bronchitis or emphysema, the airflow obstruction is generally progressive, may be accompanied by airway hyperreactivity and may be partially reversible”
-American Thoracic Society (ATS 1995) article

Latest Report:

According to Forey, Thornton, and Lee(2011,p.1)“Smoking is a major determinant of Chronic Obstructive Pulmonary Disease”. The recent findings from research estimate that COPD is the third leading cause of death worldwide. In 2019, COPD killed 3.23 million people, with 90% of the deaths occurring in subjects aged below 70 years old from low- and middle-income countries. Seventy percent of such cases are reported in industrialized nations due to tobacco smoking(WHO,2023).

Area for health promotion

According to Smith and Brown(2023,pp.78-82) smoking as a cause of COPD is a key focus of health promotion for a variety of reasons:

Prevalence and impact: COPD is one of the leading causes of morbidity and mortality worldwide; smoking is the major risk factor. The key to health promotion is increasing awareness that smoking is directly linked to COPD and that it is a preventable disease.

Behavioral risk factor: Smoking is a modifiable behavioral risk factor for COPD. Thus, health promotion activities would focus on smoking cessation programs for the reduction of the incidence of COPD and for improving overall respiratory health

Statistics

In the UK, as of 2022, 12.9% of adults are smokers, with a breakdown of 14.6% of men and 11.2% of women, totaling approximately 6.4 million people. In 2022, the smoking rates for adults in the UK were: England: 12.7% , Wales: 14.1% ,Scotland: 13.9%, Northern Ireland: 14.0%
Since 2011, the most significant decline in smoking prevalence has occurred among individuals aged 18 to 34. Those aged 25 to 34 continue to have the highest smoking rates, at 16.3%, which equates to about 1.4 million people. In the UK, over 1 in 5 people (22.8%) in routine and manual jobs smoke, compared to less than 1 in 10 (8.3%) in managerial and professional roles(ash.,2023).

Determinants of Health:

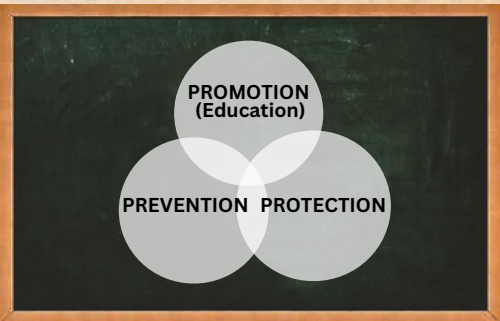
Chronic Obstructive Pulmonary Disease (COPD) is a multifaceted condition influenced by various determinants of health. Smoking is the primary cause, but other factors also play a significant role.

- **Social Determinants**
Socioeconomic Status: Lower socioeconomic status is associated with higher smoking rates and increased vulnerability to COPD due to factors like limited access to healthcare, poor living conditions, and occupational hazards(Bloomsbury,2015).
- **Environmental Determinants**
Air Pollution: Long-term exposure to air pollutants, including particulate matter and chemical irritants, exacerbates the risk and progression of COPD in smokers(WHO,2018).
- **Behavioral Determinants**
Smoking Habits: The duration and intensity of smoking directly correlate with the severity of COPD(NICE,2010).
- **Cultural Determinants**
Cultural Norms: Societal attitudes towards smoking and health-seeking behaviors impact the prevalence and management of COPD(Journal of Tobacco Research,2016).
- **Psychological Determinants**
Mental Health: Stress, anxiety, and depression can influence smoking behavior and COPD management(American Journal of Respiratory and Critical Care Medicine,2013).

Promote Health and Wellbeing regarding COPD in adults:

- Awareness campaigns:** This change will be realized through increased awareness among the public on risk factors for, symptoms, and early detection of COPD by mounting media campaigns and community outreach(GOLD,2023).
- Smoking Cessation Programs:** These should be implemented to help people quit smoking, which is the major cause of COPD; this shall be through counseling, medications, and support groups(NICE,2018).
- Telehealth Services:** Increase access to telehealth for remote monitoring and consultation, in particular, in locations that are rural or underserved(WHO,2016). According to Effing, Vercoulen and Bourbeau (2016,pp.46-54) “it reflects the importance of the role of patient education and self-management strategies in improving the quality of life and reducing the number of hospital admissions for COPD patients”.

Tannahill's (1985) Health Promotion Model



Why This Model ?

By applying Tannahill's Health Promotion Model, health professionals and policymakers can take a comprehensive and multi-faceted approach to addressing smoking as a cause of COPD. Through targeted health education, robust prevention strategies, and protective measures, the model supports efforts to reduce smoking prevalence, prevent COPD, and improve overall respiratory health

Health Education

- Public Awareness Campaigns:** Develop campaigns using various media channels (e.g., television, social media, posters ,leaflets) to educate the public about the risks of smoking and its direct connection to COPD. Highlight that COPD is preventable and that smoking cessation can significantly reduce risk(WHO,2021).
- Educational Workshops:** Conduct workshops in community centers, workplaces, and schools to provide detailed information about COPD, the dangers of smoking, and the benefits of quitting. Include personal stories from individuals who have successfully quit smoking and improved their respiratory health(WHO,2021).

Health Prevention

- Smoking Cessation Programs:** Offer comprehensive cessation programs that include counseling, nicotine replacement therapies, medications, and support groups. Make these programs easily accessible to all individuals, especially in high-risk populations(Centers for Disease Control and Prevention,2020).
- Policy Advocacy:** Advocate for public health policies that discourage smoking, such as increasing taxes on tobacco products, implementing plain packaging, banning tobacco advertising, and enforcing age restrictions on tobacco sales(Centers for Disease Control and Prevention,2020).

Health Protection

- Environmental Changes:** Promote the development of smoke-free environments in communities, including parks, restaurants, and public transportation. Encourage employers to establish smoke-free workplace policies(WHO,2019).
- Economic and Social Support:** Provide resources and support for individuals attempting to quit smoking. This could include financial incentives, access to affordable cessation resources, and community support networks to help individuals maintain smoke-free lives(WHO,2019).